



SUDAN HEALTH

Sudan Health Sector Quarterly Bulletin (1st Quarter, January – March 2016)

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Key figures

Population:	38.4 M
People in Need of humanitarian assistance (all actors):	5.8 M

Health Sector

People in Need (PIN):	3.9 M
People targeted:	3.4 M

Health, PIN by category

IDPs:	2.2 M
Residents:	1.2 M
Refugees:	0.3 M
Returnees:	0.1 M

Funds and funding

Total estimated budget	US\$ 60M
SHF (Sudan Humanitarian Fund) 2016 allocation	US\$ 3.65M

Introduction

The Health Sector in conjunction with Sudans' Federal Ministry of Health (FMoH) and humanitarian actors have identified 3.9 million people to be in need of health services, 53% of whom are children. This includes 57% IDPs, 31% refugees, 9% residents and 3% returnees.

The sector strategy aims to improve access to essential health services for crisis-affected people. This will be achieved by improving primary healthcare services including maternal and child healthcare, ensuring adequate and equitable access to clinical and public health interventions and improving referral systems in high priority localities. In particular, the sector will support the delivery of a standardized primary healthcare package for

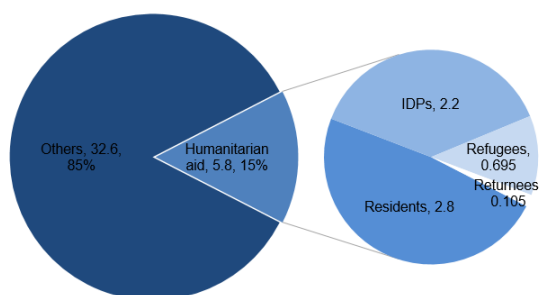
women, men, boys and girls that includes outpatient consultations, free of charge or affordable medicine, immunizations and treatment of endemic and chronic diseases. These services will be provided through existing health facilities or where health units do not exist or are insufficient, through mobile clinics, particularly for newly displaced communities.

The sector has targeted a total of 3.4 Million people, broken down as 2,225,000 vulnerable IDPs (protracted and new) in camps and gatherings and 975,000 vulnerable host communities, 75,000 returnees, and 125,000 refugees with a special focus on women and children.

Health Sectors' key objectives

1. Provide PHC services including referral services for people affected by conflicts and natural disasters.
2. Strengthen the capacities to prepare, detect, and respond promptly to public health risks at federal state and locality levels.
3. Ensure maternal and child health services for the reduction of maternal and child mortality and morbidity amongst vulnerable population.

Caseload of Sudan in million people



Sudan Humanitarian Fund 2016

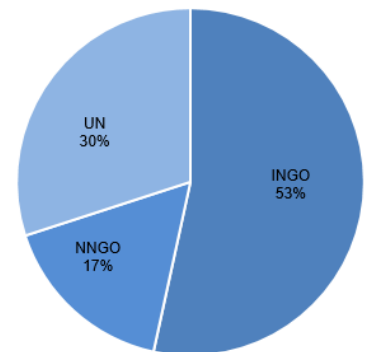
Sudans' Health Sector remains consistently underfunded. The health sector which is led by the Ministry of Health and World Health Organisation (WHO) is composed of more than 30 partner organisation aims to provide health services to 3.4 million vulnerable people this year.

The strategic review Group (SRG) of the sector reviewed 24 concept notes and recommended 11 for funding through the Sudan Humanitarian Fund (SHF) first standard allocation. The sector received US\$ 3.65M from the SHF allocation.

As the pie-chart to the right shows, the majority of the funding is to be channelled through well placed International Non-governmental Organisations (INGOs). With the United Nations (UN) organisations taking 30% and National Non-governmental organisations taking 17%.

The UN (WHO, UNICEF and UNFPA) mainly undertake purchasing and supplies of medicines and training of medical personnel.

Pie of funds allocation in the SHF by organization type.



Org. Type	Amount million (US\$)
INGO	1,949,526
NNGO	605,474
UN	1,095,000
Total	3,650,000

Viral diseases still a challenge ...

Dengue fever ...

The Health Sector, through its partners continue to identify and treat viral diseases in the community. During the reporting period, a twelve year old boy was admitted after complaining of headache and fever. He was also bleeding from the nose and mouth.

Health workers at the facility performed tests and the child was diagnosed with Dengue fever, one of the viral diseases in Sudan.

The boy was successfully treated and four days later discharged. His parents were very appreciative of the work being done by Save the Children and the Health Sector.

During treatment



After treatment



Partners' success stories...

Health facility rehabilitation and stocking by GOAL



GOAL with OFDA funds and in collaboration with the Ministry of Health have supported the opening of three new basic health units in Kutum and Al Waha localities in North Darfur in the areas of Frok, Amou and Guba. .

Existing health clinic structures were rehabilitated by GOAL and the community in each area, equipped with basic equipment and pharmaceutical supplies and staffed with support from Ministry of Health (MoH) at state and locality levels.

Since 2004, GOAL has implemented multi sector integrated humanitarian programming incorporating Primary Health Care (PHC), nutrition, informal education and Water and Sanitation (WASH). With the addition of these three health units, GOAL is now supporting 16 health facilities in North Darfur serving a catchment population of 168,671 funded by OFDA, ECHO, Irish Aid and SHF.



Distribution of feminine hygiene kits by IOM

In order to encourage women to take more ownership of their health, IOM distributed 5,000 feminine hygiene kits to the most vulnerable girls and women of reproductive age in El Sereif Camp South Darfur. Prior to distributing these kits, a sensitization exercise took place where basic education regarding reproductive health was provided. Established in 2014, the El Sereif clinic has been providing primary health care (that includes maternal care consisting of antenatal care, assisted deliveries, and urgent referrals for obstetric emergencies).



Building the capacity of health workers by UNICEF

With joint support from UNICEF and the European Union (EU), 93 community midwives from South and East Darfur States have completed their midwifery training in Omdurman. The graduation ceremony was attended by the Federal Minister of Health, the UNICEF Representative, the Wali of East Darfur State and other government partners. The midwives will return to their respective states to provide services to mothers in their communities and contribute to the reduction of maternal mortality. Additional 200 students are expected to graduate by mid-2016.



House-to-house visit by Save the Children



While on a routine House-to-house visit, a Community Health Worker employed by Save the Children (SC) discovered a child who had inflamed and fevered wounds. The child was suffering in this state for three months. His parents, when asked indicated that they thought the child was suffering from a spider bite and thus were treating him with tradition herbs.

The SC staff advised the parents to take the boy to the nearest health facility, an advice they heeded. At the hospital, the boy was examined by a doctor employed by SC. The child was admitted, treatment provided and has since been discharged after making a full recovery.

Health Sector Coordination and Monitoring

Together with FMoH, WHO steers Sudans' Health Sector. In the reporting period, the sectors' response plan for HRP was finalised and projects recommended for SHF 2016 funding.

At national level, WHO and FMoH held partner co-ordination meetings on a monthly basis to keep abreast with the recent health issues and response to new displacements and refugee influx.

At state level, these meetings were held on a weekly basis and partners updated the forum with on-going responses to Viral Haemorrhagic Fevers (VHF), measles and whooping cough cases amongst others.

WHO also participated in humanitarian coordination meetings at both national and state levels i.e. Humanitarian Country Team and the Inter Sector Coordination meetings.

Bi-weekly meetings were also held with the Nutrition and WASH sectors for inter-sector projects.

WHO with SMOH conducted field visits to monitor the quality of CHF health projects. The monitoring noted staff and management difficulties and drug supply system challenges. It was recommended that partners inform MoH and WHO of such difficulties whenever faced with such bottlenecks early.



Main activities

Health facility rehabilitation and medical supplies

WHO provided 12 Inter-agency Emergency Health Kits, assorted drugs and medical supplies to clinics managed by NNGOs, SMOH and MSF-E. These will serve an estimated 65,000 beneficiaries, most of whom are recently displaced persons and vulnerable returnees as part of the Jabel Marra Response. In addition, 122 Rapid Response Kits, meant for 366,000 beneficiaries were distributed to ARC, CIS, IMC, SC Sweden and World Relief Worldwide in Darfur region. Two trauma kits meant for 400 patients were given to SMOHs of Western and Central Darfur states.

WHO rehabilitated a 12 beds isolation/treatment centre in Ed Daien Teaching Hospital.

Capacity building of medical personnel

During the reporting period, 1,284 health workers and community volunteers received training with WHO support. The training programme included Training of Trainers (TOTs) on Integrated Management of Childhood illness (IMCI), two trainings on IMCI for medical doctors, one for medical assistances and nurses.

In addition, the SMOH and WHO conducted a training session on Rapid Response for 21 medical doctors in South Darfur.

Other trainings conducted by MoH and WHO included Rational Drugs Use and Antimicrobial Policy and Quality Assurance.

Immunization

MoH and MSF with WHO and UNICEF support completed immunization campaigns against measles and polio in North Darfur. Vitamin A was also provided to children below 15 years the coverage of polio and Vitamin A is now 69% and 70% respectively.

Kutum Midwifery School

With funding from SDC (Swiss Agency for Development and Cooperation), WHO and MoH established a midwifery training school in Kutum. The objective of this project is to contribute to the reduction of Maternal Mortality Rates (MMR) through addressing the shortage and inequitable distribution of midwives in remote locations. Health Sector partners have consistently reportedly on the limited distribution of medical personnel in remote locations despite of the fact that many graduate annually. Establishing this training facility in such a remote location and training local girls in Kutum and the neighboring nomadic communities of Umbarro, Elwaha, Elteena, Malha, Kabkabia and Saraf Omra will increase their retention and reduce the cost of bringing personnel from other locations.

Construction of the hardware components i.e. classrooms, teaching laboratory, dormitories, kitchen, recreational area, storage areas, showers, toilets and fencing on the land provided by SMOH is already complete. Kutum local authorities are in the process of connecting water and electricity to the facility. However, WHO committed its own funds to this initiative to procure and install solar system and water tanks to keep the facility running in the face of constant electricity and water cuts.

The procurement of furniture and equipment worth more than USD 128,360 for the school is in process almost entirely covered by funds from WHO. Sister Margaret made an in-kind donation of desks and chairs worth (USD 3,395).

The Academy of Health Sciences (AHS) will take the full responsibility of providing teaching materials and manuals with WHO providing library trunks with manuals on emergency.

AHS and MoH have already identified teaching and support staff to run the facility and will cover costs, 50% in the first year, 70% in the second and from the third year, all cost related to running the facility. WHO and partners will cover the remaining percentages in first and second years.



Upgrading and rehabilitating health facilities in return sites

With funding from Qatar Government, through UNDP under Darfur Reconstruction Project; UN Habitat, UNFPA, UNICEF and WHO will construct / rehabilitate or up-grade 30 health facilities in Darfur states. The project is estimated to reach more than 2.9 million people in the region.

FMoH, SMOHs, SMPUDs, DRA and AHS were key partners in identifying the facility and kind of activity to be done in the health facilities.

The project activities include rehabilitation and upgrading using Stabilized Soil Blocks (SSBs) and solar panels to minimize the impact on the environment and promote sustainability. It also involves training of personnel in different categories and procurement and delivery of medical equipment for these 30 targeted facilities as per MoH standards.

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